

1. Administrative & Study Identification

Official Study Title: A Phase III, Multicenter, Randomized, Double-Masked, Active Comparator-Controlled Study to Evaluate the Efficacy and Safety of Faricimab in Patients With Choroidal Neovascularization Secondary to Pathologic Myopia **Study Title:** A Study to Evaluate the Efficacy and Safety of Faricimab in Patients With Choroidal Neovascularization Secondary to Pathologic Myopia **Short Title/Acronym:** POYANG **Protocol Number:** CR44829 / 2023-506707-25-00 **NCT Number:** NCT06176352 **Posting ID:** 219075524383025253 **Sponsor Name / Company:** Hoffmann-La Roche / Roche **Investigational Product:** Faricimab (Vabysmo) 6.0 mg **Phase of Development:** PHASE3 **Medical Monitor:** Hoffmann-La Roche
Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of faricimab (6.0 mg) compared with ranibizumab (0.5 mg) in patients with myopic choroidal neovascularization (CNV). **Secondary Objectives:** To evaluate long-term safety, change in central subfield thickness (CST), and the drying of retinal fluid from baseline over time.

2.2 Background & Rationale Pathologic myopia can lead to the extreme stretching and thinning of the eyeball, causing damage to the retina and resulting in choroidal neovascularization secondary to pathologic myopia (mCNV). In mCNV, abnormal, leaky blood vessels grow at the back of the eye, causing rapid vision loss. While current standard treatments include anti-VEGF therapies like ranibizumab, they often require frequent injections. Faricimab is a bispecific antibody designed to block two signaling pathways (Ang-2 and VEGF-A), potentially offering improved stabilization of blood vessels, better fluid drying, and a reduced treatment burden for patients with mCNV.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator-controlled (ranibizumab 0.5 mg) **Blinding:** Double-masked (Double-blind) **Randomization:** Randomized **Phase ID:** PHASE3 **Trial Status:** ACTIVE_NOT_RECRUITING (Status Description: Active but not currently recruiting) **Criteria Type:** PHASE_REQUIREMENT (Criteria Text: Trial must be in PHASE3)

3.2 Planned Enrollment & Duration Targeted Enrollment: 280 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 06-Mar-2024 **Estimated Primary Completion:** 11-Sep-2025

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Treatment-naïve choroidal neovascularization (CNV) secondary to myopia. 2. Diagnosis of active myopic CNV in the study eye: - Presence of high myopia, worse than -6 diopters of spherical equivalence. - Antero-posterior elongation measurement greater than or equal to 26.0 mm. - Presence of posterior changes compatible with pathologic myopia (e.g., tessellated fundus, lacquer cracks, etc.). - Presence of active leakage from CNV on FFA (determined by Central Reading Centre [CRC]). - Presence of intraretinal or subretinal fluid or increase of CST on OCT (determined by CRC). 3. BCVA of 78 to 24 letters, inclusive (20/32 to 20/320 approximate Snellen equivalent), using the Early Treatment Diabetic Retinopathy Study (ETDRS) protocol on Day 1. 4. Overtly healthy as determined by medical evaluation that includes medical history, physical examination, and laboratory tests. 5. Ability to comply with the study protocol, in the Investigator's judgment. 6. Other protocol-defined inclusion criteria apply.

4.2 Exclusion Criteria 1. Any major illness or major surgical procedure within 1 month before screening. 2. Pregnancy or breastfeeding, or intention to become pregnant during the study or within 3 months after the final study treatment administration. 3. Uncontrolled blood pressure (systolic >180 millimetres of mercury [mmHg], diastolic >100 mmHg). 4. Stroke (cerebral vascular accident) or myocardial infarction within 6 months prior to Day 1. 5. History of systemic or ocular disease that would contraindicate treatment with the investigational drug or comparator. 6. Uncontrolled glaucoma in study eye. 7. Any prior or concomitant treatment for CNV or vitreomacular-interface abnormalities, including, but not restricted to, intravitreal, periocular or laser interventions in study eye. 8. Prior or concomitant periocular or intravitreal pharmacological treatment, including anti-VEGF medication, for other retinal diseases (e.g. geography atrophy, nAMD, DME etc.) in study eye. 9. Other protocol-defined exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: 6.0 mg faricimab versus 0.5 mg ranibizumab administered at a pro-re-nata (PRN) dosing regimen after an initial active IVT treatment administration at randomization (Day 1). Treatments are evaluated and administered every 4 weeks (Q4W) as needed. **Route of Administration:** Intravitreal (IVT) injection **Duration of**

Treatment: PRN treatment up to Week 44, with final follow-up at Week 48 (11 months of active treatment period).

5.2 Active Comparator Drug Metadata - Drug Name: ranibizumab (Drug Preferred Name: ranibizumab) - **ATC Code:** S01LA04 (ATC Codes List: S01LA04) - **RxNorm Code:** [Empty] - **Trade Names:** Lucentis, Susvimo, Cimerli, Byooviz - **EMA URLs:** - https://www.ema.europa.eu/en/documents/product-information/byooviz-epar-product-information_en.pdf - https://www.ema.europa.eu/en/documents/product-information/lucentis-epar-product-information_en.pdf - **Drug Semantic Type:** [Empty]

5.3 Concomitant & Prohibited Therapy Permitted: Standard ophthalmic drops or therapies not affecting the posterior segment of the study eye. **Prohibited:** Concomitant anti-VEGF medications or any other periocular/intravitreal interventions (e.g., laser treatments) in the study eye.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to -1	Informed Consent, Medical History, Ophthalmic Exam, FFA/OCT Imaging
Baseline	Day 1	Randomization, ETDRS BCVA Baseline, First IVT Dose
Interim	Weeks 4 to 44	Q4W Visits: BCVA assessment, OCT, Safety evaluation, PRN IVT Treatment if criteria met
End of Study	Week 48	Final Ophthalmic Exam, Final BCVA and CST Assessments, AE Tracking

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Change from Baseline in Best-Corrected Visual Acuity (BCVA) Averaged Over Weeks 4, 8, and 12.

7.2 Secondary Outcomes 1. Change from Baseline in BCVA Over Time. 2. Percentage of Participants Gaining ≥ 15 Letters in BCVA from Baseline Averaged Over Weeks 4, 8, and 12. 3. Percentage of Participants Gaining ≥ 15 Letters in BCVA from Baseline Over Time. *(Exploratory structural metrics also include change in central subfield thickness (CST) and absence of retinal fluid measured by OCT over time, along with overall incidence and severity of AEs.)*

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of ocular (study eye and fellow eye) and systemic non-ocular adverse events at every Q4W visit. **Serious Adverse Events (SAEs):** Standard expedited reporting within 24 hours to the sponsor and applicable regulatory bodies. **Data Monitoring Committee (DMC):** Independent

external DMC employed to monitor masked safety data throughout the trial phase.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy (non-inferiority). Safety Set for all participants receiving at least one dose of the study drug.

Sample Size Justification: $N=280$ appropriately powered ($\alpha = 0.05$, $Power \geq 80\%$) to test the non-inferiority hypothesis of faricimab compared to the active comparator (ranibizumab).

Handling of Missing Data: Mixed-Model for Repeated Measures (MMRM) and Multiple Imputation (MI) methodologies for visual acuity and anatomical continuous variables.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: Conducted in full accordance with Good Clinical Practice (GCP) guidelines and the Declaration of Helsinki.

Monitoring Plan: Regular site monitoring; strict masking protocols to ensure BCVA examiners and patients remain blinded to the allocated treatment arm. **Audit Trail:** Utilizing a Central Reading Centre (CRC) for unbiased, independent assessments of Fundus Fluorescein Angiography (FFA) and Optical Coherence Tomography (OCT).

11. Study Identifiers & Governance

Primary ID: {{CR44829}} **Registry Number (NCT):** {{NCT06176352}}
Posting ID: {{219075524383025253}} **Sponsor/Lead Organization:** {{Hoffmann-La Roche / Roche}} **Study Title:** {{POYANG}} **Official Regulatory Title:** {{A Phase III, Multicenter, Randomized, Double-Masked, Active Comparator-Controlled Study to Evaluate the Efficacy and Safety of Faricimab in Patients With Choroidal Neovascularization Secondary to Pathologic Myopia}} **Status Name:** {{ACTIVE_NOT_RECRUITING}} **Source Level:** {{5}}

12. Clinical Framework & Terminology Metrics

12.1 Disease & Primary Condition Targets Disease Areas:

{{Choroidal Neovascularization Secondary to Pathologic Myopia}}

Preferred UMLS Name: {{Myopic Choroidal Neovascularization}}

Diagnosis Threshold: {{High myopia worse than -6 diopters, Antero-posterior elongation ≥ 26.0 mm}} **Medical Methodology:** {{Intravitreal Anti-VEGF / Bispecific Antibody Therapy}}

12.2 Incidental / System Metadata Codings (Database Artifacts)

The following standardized codings are linked to the structural

metadata record format, detailing mapped classifications: - **Name:** B-cell Non Hodgkin Lymphoma - **Preferred Name:** Indolent B-Cell Non-Hodgkin Lymphoma - **Semantic Type:** Neoplastic Process - **Definition:** A B-cell non-Hodgkin lymphoma with an indolent clinical course. Representative examples include grade 1 and grade 2 follicular lymphoma, marginal zone lymphoma, and lymphoplasmacytic lymphoma. - **MeSH Code:** D008182 (List of other MeSH codes: [Empty]) - **HPO Codes:** [Empty] - **SNOMED ID:** 92000000 - **SNOMED Hierarchy:** 128361009 | Non-Hodgkin lymphoma | Non-Hodgkin lymphoma (disorder)

13. Study Procedures & Methodology

Management Pathway: {{Intravitreal injection PRN dosing regimen based on prespecified retreatment criteria}} **Education Protocol (HCP):** {{ETDRS protocol training and masked IVT administration guidelines}} **Education Protocol (Patient):** {{Vision change symptom tracking and study visit adherence}} **Quality Audit Mechanism:** {{Central Reading Centre (CRC) validation for FFA and OCT grading}}

14. Observation & Follow-Up Schedule

Total Duration: {{48 weeks}} **Onsite Visit Cadence:** {{Day 1, and every 4 weeks (Q4W) up to Week 48}} **Remote Monitoring Frequency:** {{As needed for ongoing safety/AE tracking}} **Checklist Review Interval:** {{Every 4 weeks (Q4W) prior to PRN dosing decisions}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				